AC-07 Adult Monomorphic Wide-Complex Tachycardia

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for adults age 16 years or older with a pulse and a monomorphic wide-complex tachycardia. Assume ventricular tachycardia when uncertain.

Wide-complex tachycardia pathway

1. Confirm pulse + rhythm	QRS at least 0.12 sec, monomorphic; support airway/ventilation, oxygen if hypoxemic, monitor/pads, BP/SpO2, and IV/IO.
2. Unstable	Synchronized cardioversion 100 J biphasic, increase if unsuccessful, confirm sync before every shock, and sedate when feasible without delaying shock.
3. Stable uncertain origin	Only if regular and monomorphic: adenosine 6 mg rapid IV/IO plus flush, then 12 mg after 1-2 minutes if needed; may repeat 12 mg once.
4. Stable monomorphic VT	Amiodarone 150 mg IV/IO over 10 minutes. If instability develops, stop infusion and cardiovert immediately.
5. Persistent/recurring VT	Synchronized cardioversion 100 J for persistent VT. For recurrence, repeat amiodarone 150 mg over 10 minutes once, then infusion 1 mg/min.
6. Pulse lost	Begin CPR immediately and open AC-09 VF/Pulseless VT.

Key treatment and handoff

Item	Approved action
Do not stack antiarrhythmics	Use the defined amiodarone sequence; do not independently stack multiple antiarrhythmics.
Reassess	Post-treatment strip/12-lead, complete vital signs, early receiving notification, and prepare for recurrence or polymorphic rhythm.

Do not administer diltiazem, verapamil, metoprolol, or another AV-nodal blocker for an undifferentiated wide-complex rhythm.

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SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- Adenosine is restricted to a regular, monomorphic wide-complex rhythm. Never use it for polymorphic, irregular, or pre-excited AF/flutter.

- Do not give amiodarone for suspected torsades, a clearly prolonged-QT polymorphic rhythm, or suspected pre-excited AF/flutter.

Linked pathways

Pathway	Use when
AC-08	Polymorphic wide-complex rhythm
AC-09	VF/pulseless VT if pulse lost
AC-10	Post-resuscitation care after ROSC

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.